



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نسرين محمود ابوخرس محمد يوسف

المحافظة : القاهرة الحي / المدينة : حلوان العنوان : =

رقم البطاقة : 27611030102887 رقم التليفون : 01001700862 تاريخ تسجيل الإستمارة : 21-09-2026

العمر (تاريخ الميلاد) : 03-11-1976 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور قسم العلاج : الاورام [14163]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نسرین محمود ابوخرس محمد يوسف National ID: 27611030102887	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Request No. : 13007381
Request Date : 24/05/2025
Verification Date : 10/06/2025
Financial Category : نفقة الدولة - أورام
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Nature of Specimen: Tru-cut biopsy, collected at 22/05/2025.

- Multiple grayish white tissue cores up to 1.5 cm long, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

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- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 100% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
 - Her2: Negative membrane reaction in tumor cells (score 0).
 - Ki67: 55% nuclear reaction of tumor cells.

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History of right MRM, Right breast operative bed mass, Tru-cut biopsy:
invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).

2.

Dr.Heba Hasan

Printing Date : 12/06/2025 11:36:22

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Printing By : Mrs. Mayada Hegazy

Request No. : 13007381
Request Date : 24/05/2025
Verification Date : 10/06/2025
Financial Category : فئة المزايا - Out Patient
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

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- ¹
- Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the cell surface.

within < 10% of the invasive tumor cells. IHC 1+, as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+, based on circumferential membrane staining.

circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ghoneim
Dr. Heba Hassan.

Dr. Rana Fathy

Verified by :

Dr. Heba Hasan

Printing Date: 12/05/2025 11:36:22

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Room No. (3) - First Floor Building

MDT

Patient Name	Nesreen Mahmoud Abo Khars	Age	48 years
Sex	Female	Special habits	-ve
Family History	+ve (aunt breast cancer)	Co-morbidity	-ve
Surgical History	Tonsillectomy 26y ago Cesarian 3 times Total thyroidectomy on eltroxin	Telephone	01011144953
Menopausal Status	Pre menopausal	Marital Status	MARRIED with 3 offsprings youngest 6 years
MEDICAL ID		ID	

right breast cancer , IDC II (ER 8/8 , PR 8/8 , HER2 -ve)

Metastatic breast cancer

2009
Rt breast mass with discharge ,
Upfront Rt MRM (IDC G1 on top of fibrocystic disease, N0/33 , free surgical margin) Received adjuvant chemo (FEC x 3)Metastatic workup : free
Kept on hormonal : Nolvadex for 5 years

2018
Good pregnant , off ttt

08/2021
CT CAP
Bilateral pulmonary nodules - left hepatic focal lesions likely hemangiomas
small lytic sclerotic foci.

09/2021 Till 05/2025
Started Femara + Zoladex + Zometa/ 3 months till now .

Bone scan 10/2024
No definite osseous lesions.

02/2025
CT CAP without contrast
- Increased multiple bilateral scattered pulmonary nodules, largest 8 ml compared to 6 mm in 03/2024
-progressive course of right pleural effusion (mild)
-stationary course regarding the sclerotic osseous lesions (C7, D4,D9 and rt iliac bone small sclerotic osseous foci , L3, L4 small lytic foci -Left sacroiliitis
-Rt MRM showed suspicious solid mass BIRAD5 >> recurrence?
-no left breast suspicious findings (BIRAD 2)

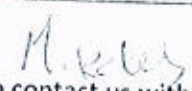
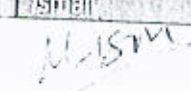
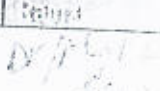
04/2025
SM
Further progression of the amount of the right pleural effusion
Otherwise, Stationary course of the rest of the study.

CT CAP with contrast
12/5/2025

Other investigations
FSH :8.7 LH :0.3 E2 :5

**MDT
DECISION**

FOR TRU CUT BIOPSY FROM OPERATIVE BED MASS
US GUIDED TAPPING OF RIGHT PLEURAL EFFUSION, CELL BLOCK
IF SAME BIOLOGY FOR ZOLADEX FASLODEX IBRANCE

Name	Prof. Axel Lebesbre	Dr / Ahmed Morsy	Dr / Mohamed Kefau	Dr / Amr Hassan	Dr / Mohamed Saeed	Dr / Mohamed
Signature						

Please feel free to contact us with any questions regarding case.
Much obliged, Dr. Ahmad Morsy, Ahmed.morsy@elevatehospitals.com +20111-111-8879

Patient's Name: Nesrin mahmoud abokhers
Date: 2026-04-22
Referred by: Prof.Dr. cap+c
Hospital: Goustave Roussey International Egypt

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
& PELVIS REVEALED: -**

Known case of breast CA underwent right MRM. Compared to the previous study dated 8-11-2025, the current study showed:

1. Chest:

- Stationary course regarding the previously seen operative bed nodule measuring 6 X 8 mm.
- Progressive course regarding the amount of the previously seen moderate right pleural effusion as well as the underlying subsegmental compression collapse.
- Still noted few bilateral scattered pulmonary nodules currently the largest at the left lower lobe anterior basal segment measures 6 mm.
- No pathological hilar or mediastinal lymph nodes.
- Bilateral basal parenchymal bands.
- No evidence of left pleural effusion.
- No gross cardiomegaly.
- No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is of average size showing regular outline with no definite focal lesion. No intra-hepatic biliary radicles dilatation.
- Gall bladder stones are still seen, the largest measures 13 mm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Apart from stable tiny right renal angiomyolipoma.
- Normal appearance of the pancreas and both suprarenal glands.
- Regular filling of the urinary bladder showing no stones, masses or diverticular outpouchings.
- Normal appearance of the other pelvic organs.

Patient's Name: Nesrin mahmoud abokhers
Date: 2026-04-22
Referred by: Prof.Dr. cap+c
Hospital: Goustave Roussey International Egypt

- No abdominal or pelvic lymphadenopathy or collections.
- Skeletal scan show:
- Stationary course:
 - Still noted D4, D9 and right iliac bone small sclerotic osseous foci
 - L3 and L4 small lytic foci.

OPINION:

- Progressive course regarding the previously seen right pleural effusion and underlying compression collapse.
- Stationary course regarding the rest of the study.

Dr.sherif Wagdy

Dr.sherif Wagdy

Name : Nasreen Mahmoud Abokharas
Mohammed
Branch : Dokki

Date : 20-12-2025 01:32

Medical No. : 277579

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- A 49-year-old lady, with history of right breast cancer and referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- Considering the prior study dated 5/10/2024; still no abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is keeping free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient's Name: Nasrin Mahmoud Abu Khreis Muhammad
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS WITHOUT CONTRAST ADMINISTRATION REVEALED:-

- Evident right breast mastectomy with still seen operative bed focal lesion ,measuring about 8mm with no appreciable changes seen.
- Normal left breast and right axilla.
- Multiple variable sized benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 18mm.
- Right sided pleural effusion is still seen.
- Subpleural reticulations are seen in the anterior segment of right upper pulmonary lobe.
- Patchy area of consolidation seen in the posterior segment of right upper pulmonary lobe,bronchiectatic changes with thickened wall in the right lung.
- Thin linear atelectatic band is also still seen in the anterior segment of left lower pulmonary lobe and scattering in the right lung field.
- The tiny left pulmonary nodule is still seen in the anterior segment of left upper pulmonary lobe and having the same diameter of 6mm.
- No other pulmonary nodules or sizable mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No left pleural or pericardial effusions.
- Average size of the heart with normal CT of its big vessels.
- Tiny foci of osteosclerosis are still seen in DV4,DV9 and tiny lytic bony lesions in LV3,nd LV4.
- Average sized liver,measuring about 11.4 cm, showing a homogenous parenchymal attenuation pattern.
- No sizable hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- A gall bladder stone is seen,measuring about 8mm with its CT density of 543HU.
- Average sized spleen,measuring about 6.8 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

A status follow up, and comparing with the previously-done examination dated at 22/04/2026, the current study shows:

- Evident right breast mastectomy with still seen operative bed focal lesion with no appreciable changes seen herein,denoting a stable course.
- Right sided pleural effusion still seen with subpleural reticulations in the anterior segment of right upper pulmonary lobe, patchy area of consolidation in the posterior segment of right upper pulmonary lobe,bronchiectatic changes with thickened wall in the right lung ,thin linear atelectatic bands also still seen in the anterior segment of left lower pulmonary lobe and scattering in the right lung field, the tiny left pulmonary nodule also still seen in the anterior segment of left upper

Patient's Name: Nasrin Mahmoud Abu Khreis Muhammad
Date: 2026-08-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

pulmonary lobe and tiny foci of osteosclerosis still seen in DV4,DV9 and tiny lytic bony lesions in LV3,nd LV4,denoting a stable course.

- Calcular cholecystitis.

Prof/Hanan El-Ahwal.

Dr.Hanan Alahwel

Dr.Hanan Alahwel